

RIVERSIDE GENERAL HOSPITAL

Department of Internal Medicine | Inpatient Medical Record

Patient Information

Patient Name:	Rajesh Kumar Mehta	MRN:	RG-2026-004821
Age / Sex:	58 / Male	Date of Birth:	14-Mar-1968
Admission Date:	02-Aug-2026, 09:15	Discharge Date:	07-Aug-2026, 11:40
Attending Physician:	Dr. Sunita Rao, MD (Internal Medicine)	Ward / Bed:	Medicine Ward B / Bed 12

Admission Details

Chief Complaint: Progressive shortness of breath, bilateral ankle swelling, and fatigue for 5 days.

History of Present Illness: Patient presented to the Emergency Department with worsening dyspnea on exertion over the past 5 days, orthopnea (requiring 2 extra pillows), and bilateral pitting ankle edema. Reports reduced urine output over the last 2 days. No chest pain. Known hypertensive and type 2 diabetic on irregular medication compliance. No prior hospitalization for cardiac illness.

Past Medical History: Hypertension (12 years), Type 2 Diabetes Mellitus (8 years), Dyslipidemia. No known drug allergies.

Admitting Diagnosis: Acute decompensated heart failure (NYHA Class III) with poorly controlled Type 2 Diabetes Mellitus and Hypertension.

Vitals at Admission

Blood Pressure:	162/98 mmHg	Heart Rate:	104 bpm
Respiratory Rate:	26 /min	SpO2:	91% on room air
Temperature:	98.4 °F	Weight:	82 kg

Key Laboratory Results (Day 1)

Test	Result	Reference Range	Flag
Hemoglobin	11.8 g/dL	13.5 - 17.5 g/dL	Low
Fasting Blood Glucose	216 mg/dL	70 - 100 mg/dL	High
HbA1c	9.1 %	< 5.7 %	High
Serum Creatinine	1.6 mg/dL	0.7 - 1.3 mg/dL	High
BUN	38 mg/dL	7 - 20 mg/dL	High
Sodium	133 mEq/L	135 - 145 mEq/L	Low

Potassium	5.1 mEq/L	3.5 - 5.0 mEq/L	High
NT-proBNP	3,850 pg/mL	< 300 pg/mL	High
Troponin I	0.02 ng/mL	< 0.04 ng/mL	Normal

Imaging & Investigations

Chest X-Ray (Day 1): Cardiomegaly with bilateral pulmonary venous congestion and small bilateral pleural effusions, consistent with fluid overload.

Echocardiogram (Day 2): Left ventricular ejection fraction (LVEF) 35%. Global hypokinesia. Mild mitral regurgitation. No significant valvular stenosis. Findings consistent with heart failure with reduced ejection fraction (HFrEF).

Hospital Course

Patient was admitted and started on intravenous diuretics (furosemide) with strict input/output monitoring and daily weight charting. Oxygen supplementation was provided initially for hypoxia, which resolved by Day 3. Blood glucose was managed with basal-bolus insulin therapy given significantly elevated HbA1c, with plans to transition to oral antidiabetics closer to discharge. Renal function and electrolytes were monitored daily given elevated creatinine and potassium on admission, both of which trended toward normal by Day 4. Antihypertensive therapy was optimized. Patient's dyspnea, orthopnea, and lower limb edema showed steady improvement through the admission, with weight decreasing from 82 kg to 77.5 kg by Day 5. Patient was ambulating independently and tolerating a low-sodium diabetic diet by Day 6, and was deemed stable for discharge on Day 6 evening, discharged Day 7 morning after final physician review.

Discharge Medications

Medication	Dose	Frequency	Duration
Tab. Furosemide	40 mg	Once daily, morning	Ongoing - review in 2 weeks
Tab. Metoprolol Succinate	25 mg	Once daily	Ongoing
Tab. Ramipril	2.5 mg	Once daily	Ongoing
Tab. Metformin	500 mg	Twice daily, with meals	Ongoing
Tab. Atorvastatin	20 mg	Once daily, night	Ongoing
Tab. Spironolactone	25 mg	Once daily	Ongoing - monitor potassium

Discharge Condition & Follow-Up

Condition at Discharge: Stable, ambulatory, afebrile, SpO2 97% on room air. NYHA Class II functional status.

Follow-Up Instructions: Follow up with Cardiology outpatient clinic in 2 weeks with repeat renal function tests and serum electrolytes. Follow up with Endocrinology in 4 weeks for diabetes management review. Daily weight monitoring at home; report to hospital if weight increases by more than 2 kg in 3 days or if breathlessness worsens. Low-sodium, diabetic-friendly diet advised. Smoking and alcohol cessation counseling provided.

Discharge Diagnosis: Acute decompensated heart failure with reduced ejection fraction (LVEF 35%), Type 2 Diabetes Mellitus (poorly controlled), Hypertension, Stage 2 Chronic Kidney Disease (resolving acute component).

This is a synthetic, fictional medical record created for software testing purposes only. The patient, physician, hospital, and all clinical details are entirely fictitious and do not represent any real individual or institution. Intended for use as sample input to test the Clinical Discharge Summary Agent (OCR + LLM pipeline) only.